

Name	MR.JAYAKUMAR J	Patient ID	AND25290005474
Accession No.	C2529001537489	Age/Sex	50Y / Male
Ref. Doctor	CANCARE FOUNDATION	Date	11-Jul-2025

FDG PETCT SCAN WHOLE BODY WITH MRI SCREENING

Clinical History / Data:

- ❖ Known case of carcinoma stomach (May 2023).
- ❖ Post chemotherapy status (Apr 2024).
- ❖ PETCT done elsewhere (25.04.2024) – Suggestive of disease progression.
- ❖ Post chemotherapy status (Aug 2024).
- ❖ PETCT (05.09.2024) – Non-FDG avid mural thickening in the pyloric antrum of stomach. Metabolically active irregular enlarged peripyloric lymphnode - Active lymphnodal metastasis. Multiple metabolically active peripherally enhancing hypodense lesions in both lobes of liver - Active hepatic metastases - Suggestive of disease progression.
- ❖ Post 8 cycles of chemotherapy status (23.12.2024).
- ❖ PETCT (03.01.2025) – Features are suggestive of favorable response to therapy.
- ❖ Post chemotherapy status (06.02.2025).
- ❖ PETCT (18.02.2023) – Non-FDG avid persistent mural thickening in the pyloric antrum of stomach. Stable peripyloric and left gastric lymphnodes without uptake. Mild increase in size of hypodense lesions in both lobes of liver with interval increase in metabolic activity – Active hepatic metastases.
- ❖ Post 6 cycles chemotherapy status (June-2025).

Indication for the PET scan:

- ❖ For follow up

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PET-CT Conclusion:

On comparison with previous PETCT dated: 18.02.2025

- ❖ Increase in mural thickening in the pyloric antrum of stomach with appearance of increased metabolic activity – Active disease.
- ❖ Increase in size of peri pyloric and left gastric lymph nodes with appearance of metabolic activity – Active nodal metastases.
 - Appearance of metabolically active subcentimeter celiac, peripancreatic, paraaortic, retrocaval and paracaval lymphnodes.
 - Small peripancreatic lymphnode is seen infiltrating the adjacent neck of pancreas.
 - Appearance of few subcentimeter bilateral cervical, mediastinal and right hilar lymphnodes with increased metabolic activity.
- ❖ Appearance of diffuse mesenteric and omental haziness without significant metabolic activity – *Suggested follow up to rule out early metastases.*
 - Appearance of diffuse peritoneal thickening without significant metabolic activity.
 - Appearance of mild ascites
- ❖ Increase in size, number and metabolic activity of heterogeneously enhancing hypodense lesions in both lobes of liver – Active hepatic metastases.
- ❖ Appearance of small T2 heterointense lesion with restricted diffusion abutting and infiltrating the neck of pancreas with increased metabolic activity.
- ❖ Appearance of metabolically active multiple pulmonary nodules in bilateral lung fields with some showing increased metabolic activity – Active pulmonary metastases.

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- ❖ Appearance of metabolically active multiple lytic / sclerotic lesions in right ilium, sacrum, right mandibular condyle, D2, D10, D12 and L4 vertebra – Active skeletal metastases.
- ❖ No evidence of metabolically active disease anywhere else in the body.

Comments:

- PETCT features are suggestive of progressive disease.

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PETCT - PROTOCOL	
RADIO-ISOTOPE:	¹⁸ F-FDG was injected intravenously.
STUDY MODE:	PET CT-3D mode: Motion Free acquisition.
EXTENT OF STUDY:	Head to mid thigh
CONTRAST:	IV contrast given.
BLOOD GLUCOSE	132 mg / dl
Sr. CREATININE	0.6 mg / dl
SUVs Based on body weight	

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PETCT FUSION

Physiological concentration noted in the brain, myocardium, liver, gut, kidneys and urinary bladder.

PETCT abdomen and pelvis with MRI screening:

Increase in mural thickening noted in the pyloric antrum of stomach measuring ~ 28mm in length and ~13mm in thickness (Previously measured ~ 7-8 mm in thickness) with appearance of increased metabolic activity. (SUV Max = 4.9)

- Mild T2 heterointense mural thickening with restricted diffusion noted in the pylorus of stomach.
- Increase in size of peri pyloric and left gastric lymph nodes are noted, largest measuring ~31 x 20mm (Previously measured ~ 11 x 9mm) with appearance of metabolic activity. (SUV Max = 3.6)

Appearance of FDG avid subcentimeter celiac, peripancreatic, paraaortic, retrocaval and paracaval lymphnodes are noted (SUV Max = 4.8)

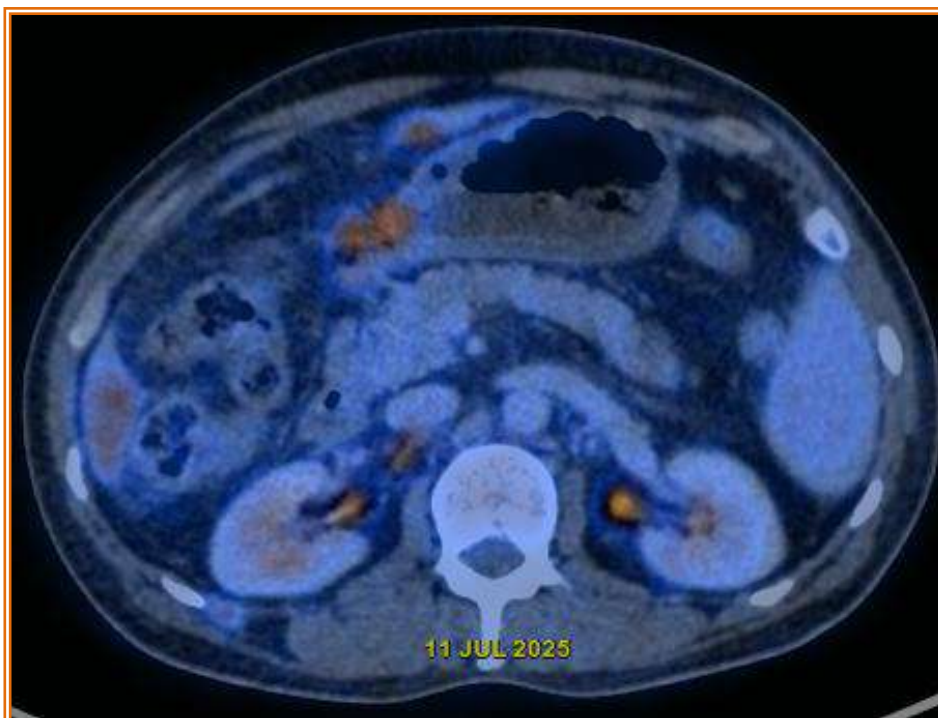
- Small peripancreatic lymphnode is seen infiltrating the adjacent neck of pancreas.

Appearance of diffuse mesenteric and omental haziness noted without significant metabolic activity.

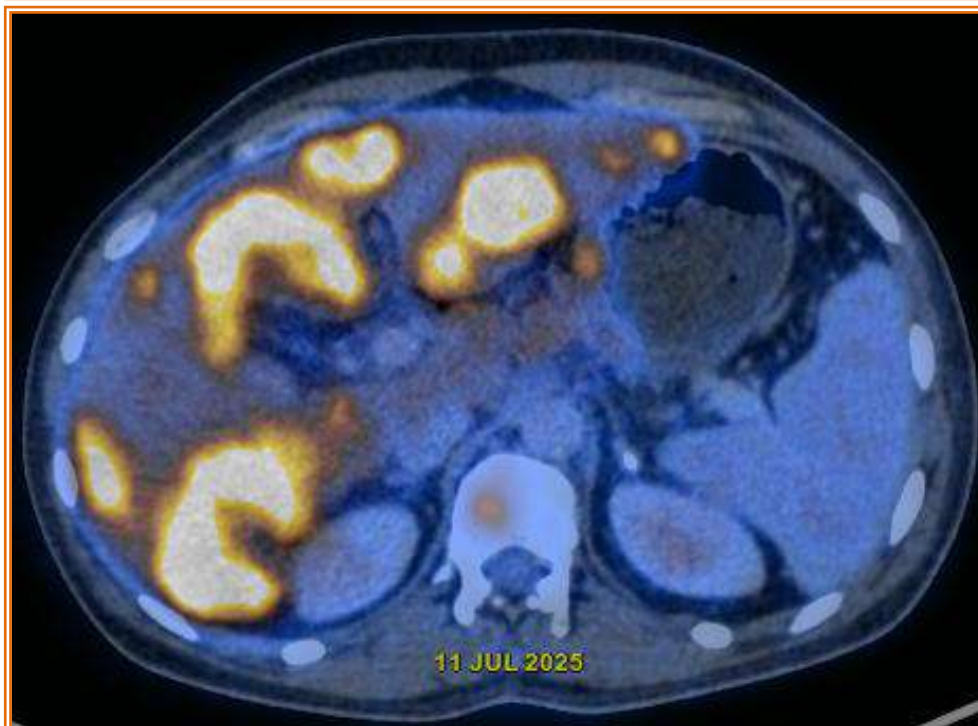
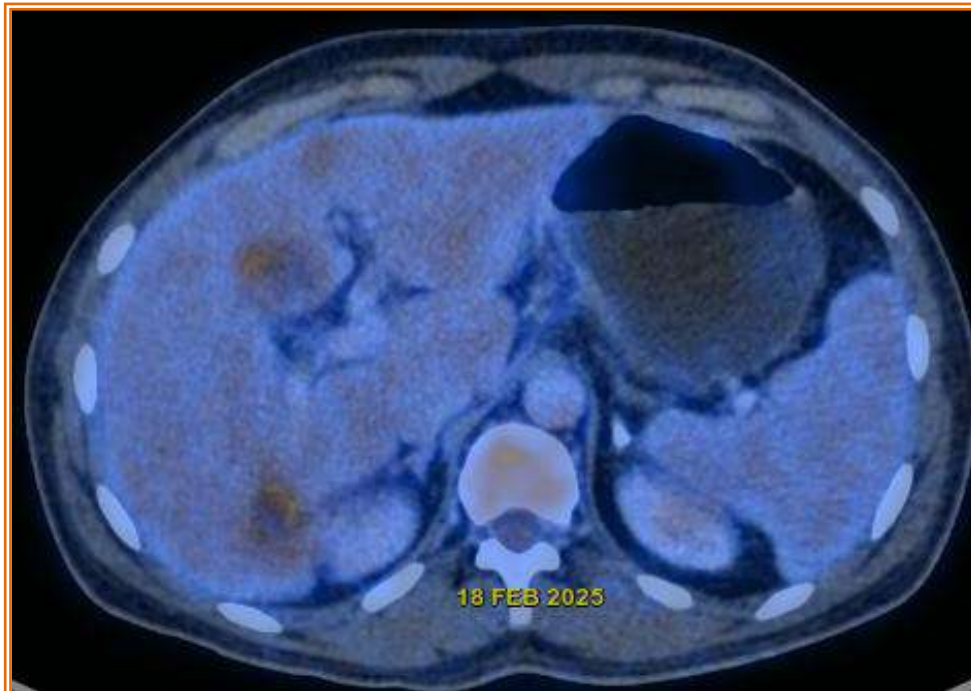
- Appearance of diffuse peritoneal thickening noted without significant metabolic activity.
- Appearance of mild ascites

G.I tract: The duodenum and proximal jejunal loops are normal in caliber. The ileum and ileo-caecal junction are normal. The colon and rectum are unremarkable.

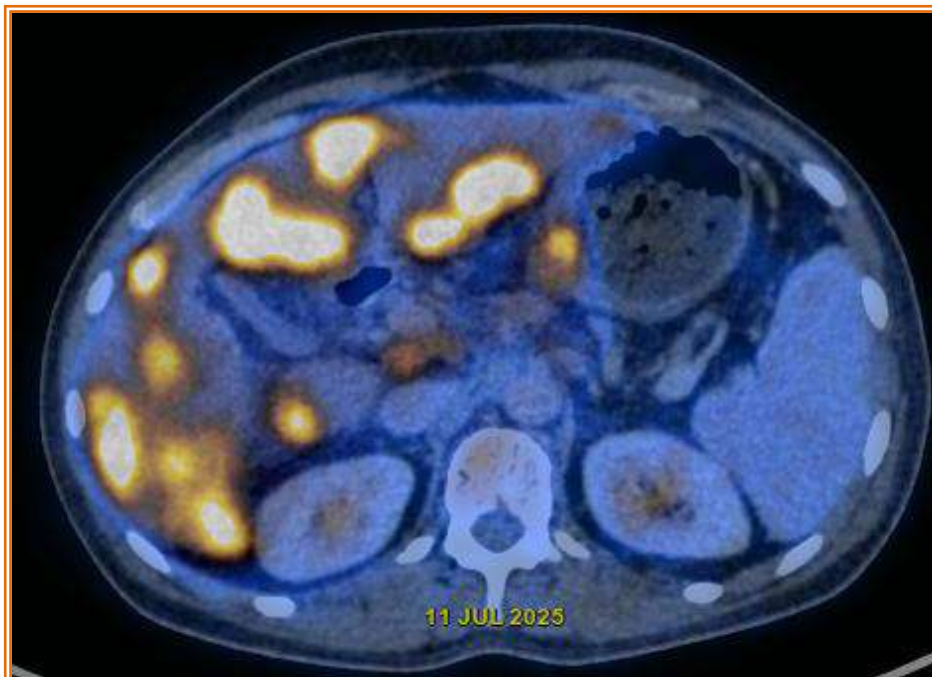
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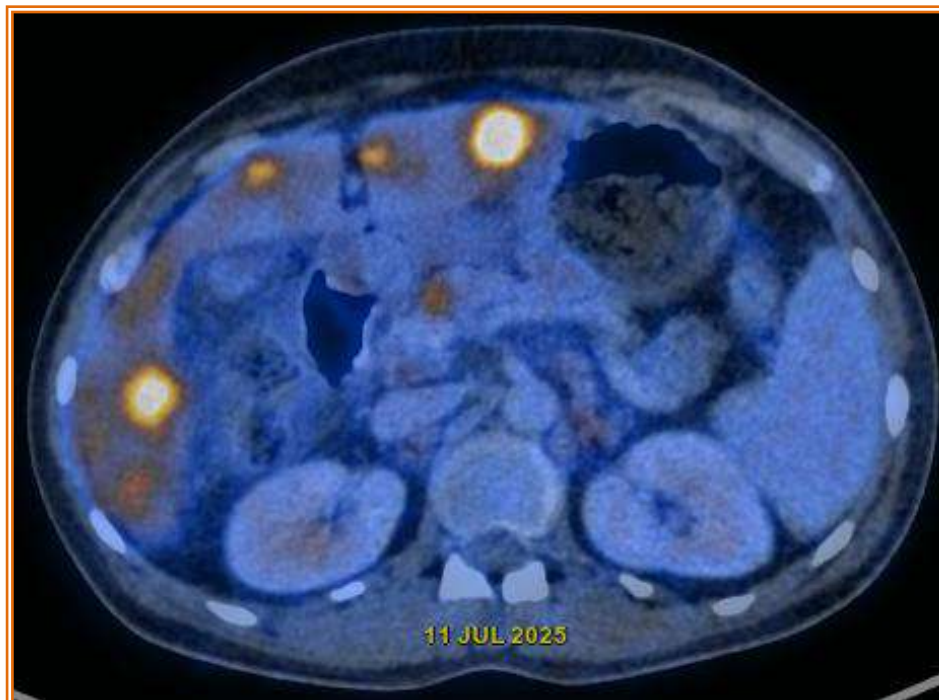
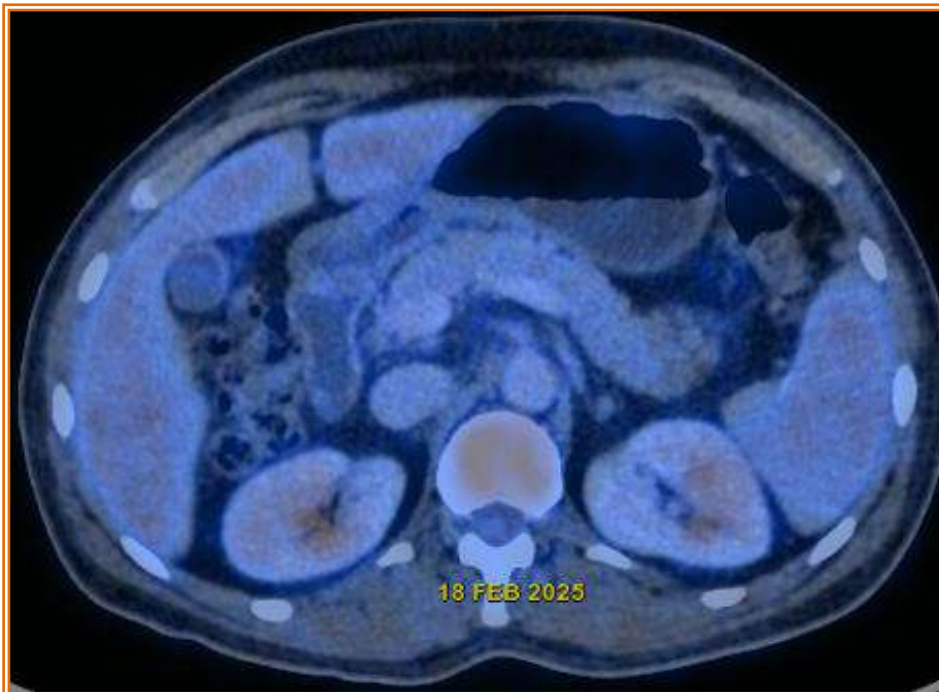
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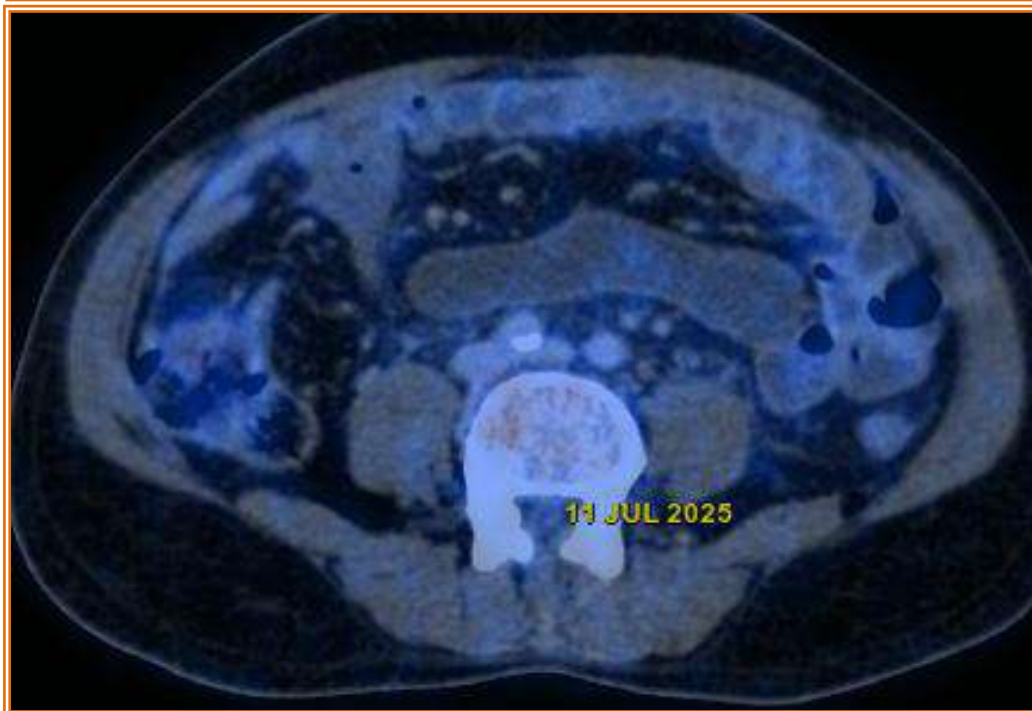
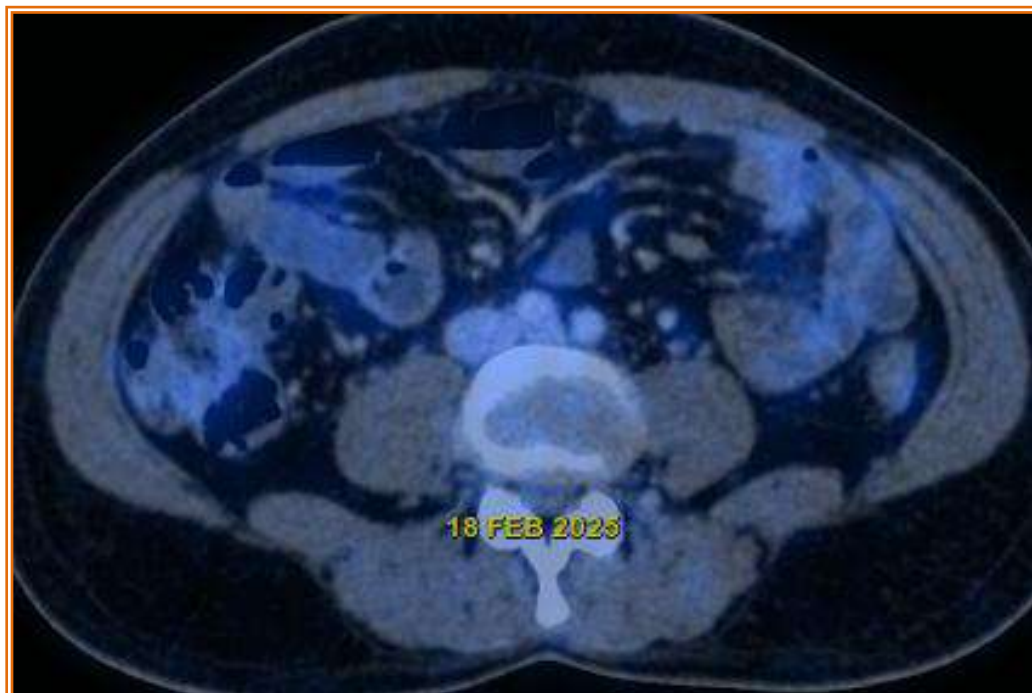
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Patchy increased metabolic activity in the ileal loop in right iliac fossa without obvious mural thickening.

Liver: **Increase in size, number and metabolic activity of heterogeneously enhancing hypodense lesions noted in both lobes of liver, largest measuring ~ 75 x 74mm in segment (Previously measured ~ 27 x 24mm) in segment VII (SUV Max = 13.4)**

- Multiple T2 heterointense lesions with restricted diffusion noted in both lobes of liver.

The porta hepatitis is normal. The intrahepatic portal venous radicals are normal. No evidence of intrahepatic biliary radicular dilatation. The hepatic veins and intrahepatic portion of inferior vena cava are normal.

Gall bladder: Is contracted.

Spleen: Normal in size, shape and attenuation values. The splenic hilum and splenic vein are normal

Pancreas: Otherwise, normal in size, shape and attenuation values. No evidence of pancreatic duct dilatation.

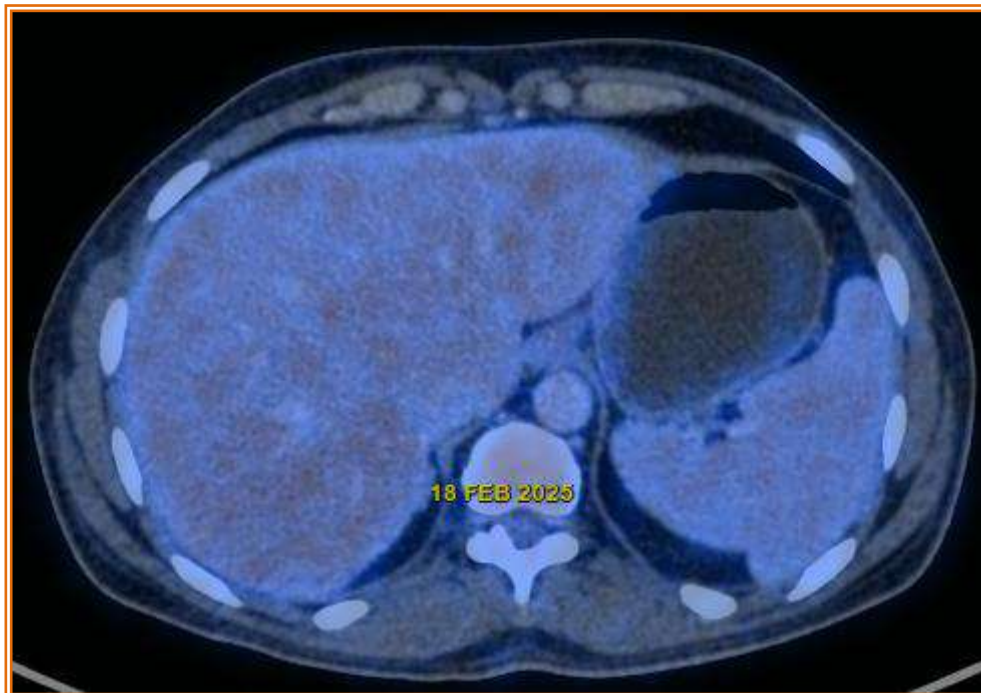
Adrenal glands: Calcifications noted in left adrenal gland. Right adrenal appears normal in size, shape and attenuations values.

Kidneys: Right kidney is normal in size and shape. The renal outlines are normal. No evidence of focal mass lesion/hydronephrosis/calculi.

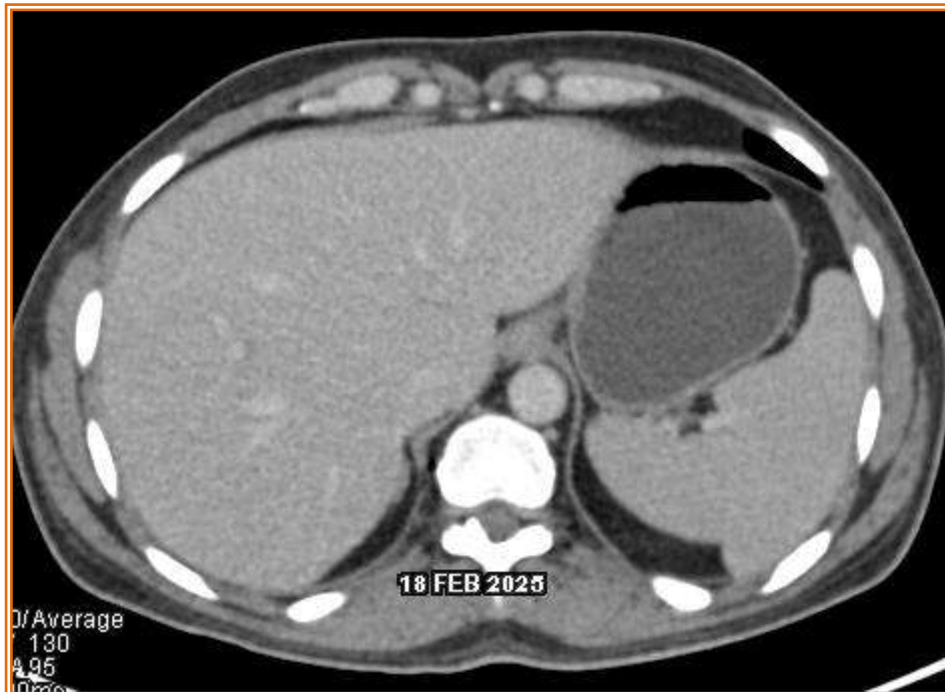
Left kidney is normal in size and shape. The renal outlines are normal. No evidence of focal mass lesion/hydronephrosis/calculi.

Retroperitoneum: The aorta and IVC appear normal.

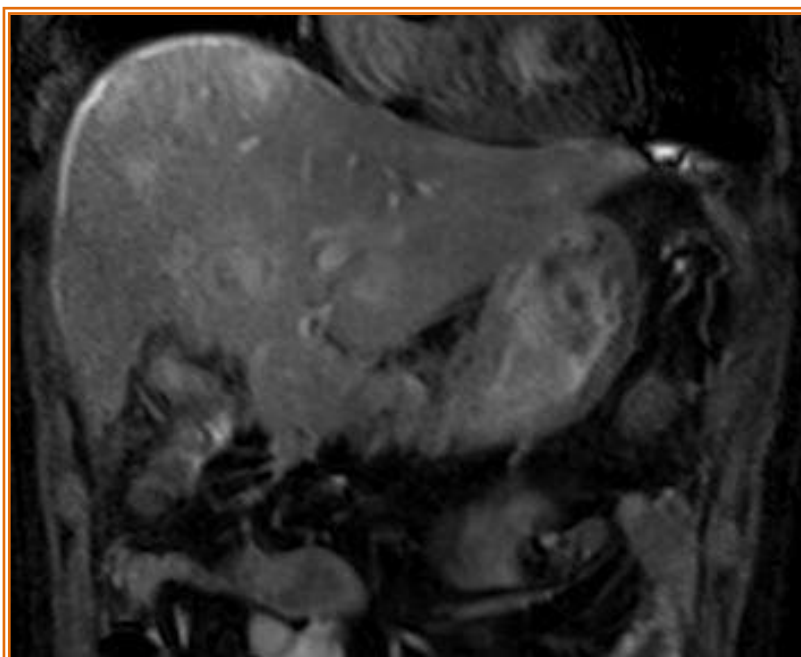
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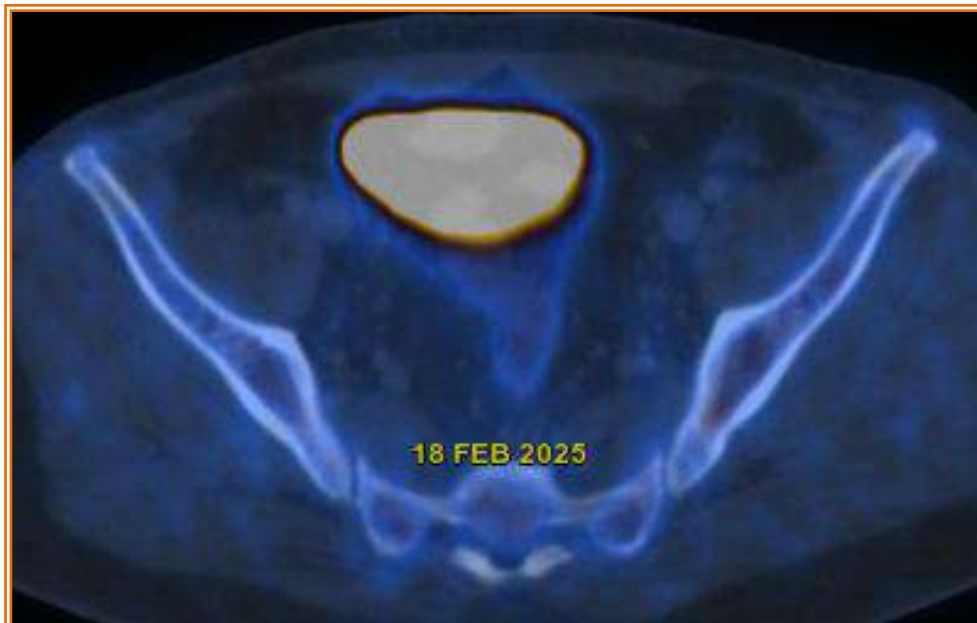
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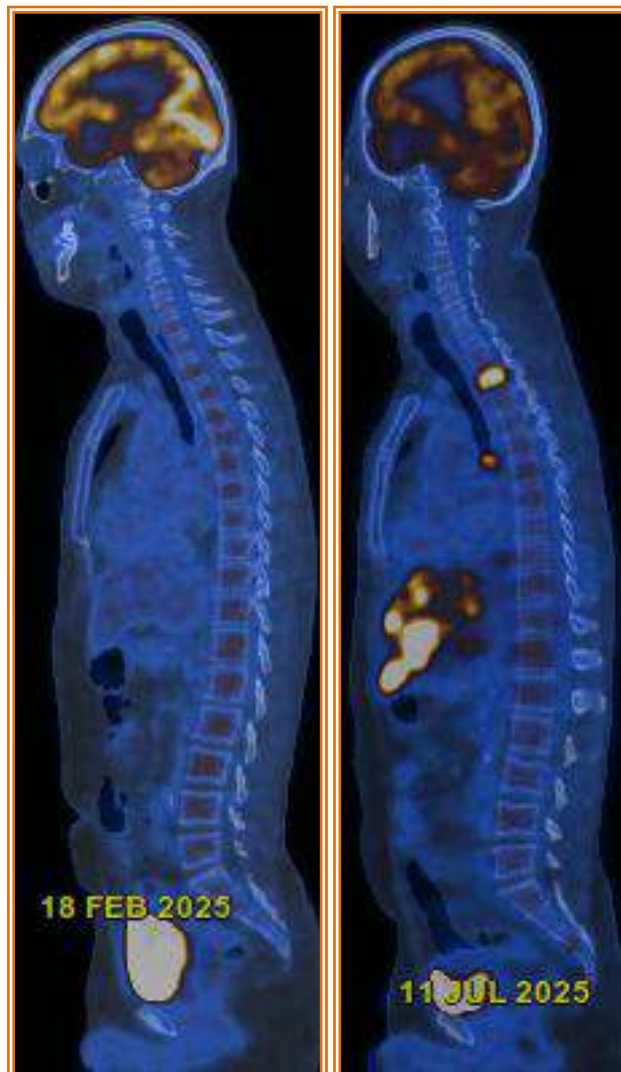
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Urinary bladder: The urinary bladder is empty

Prostate is normal in attenuation.

Appearance of FDG avid multiple lytic / sclerotic lesions noted in right ilium, sacrum, right mandibular condyle, D2, D10, D12 and L4 vertebra. (SUV Max = 6.9)

PETCT Neck:

Appearance of subcentimeter bilateral level IV and left supraclavicular lymphnodes with increased metabolic activity. (SUV Max = 4)

The oropharynx, hypopharynx, supraglottic and infraglottic larynx and upper trachea are unremarkable.

The post cricoid region appears normal.

The vocal cords and the ary-epiglottic folds appear normal.

The parotid and submandibular glands appear normal.

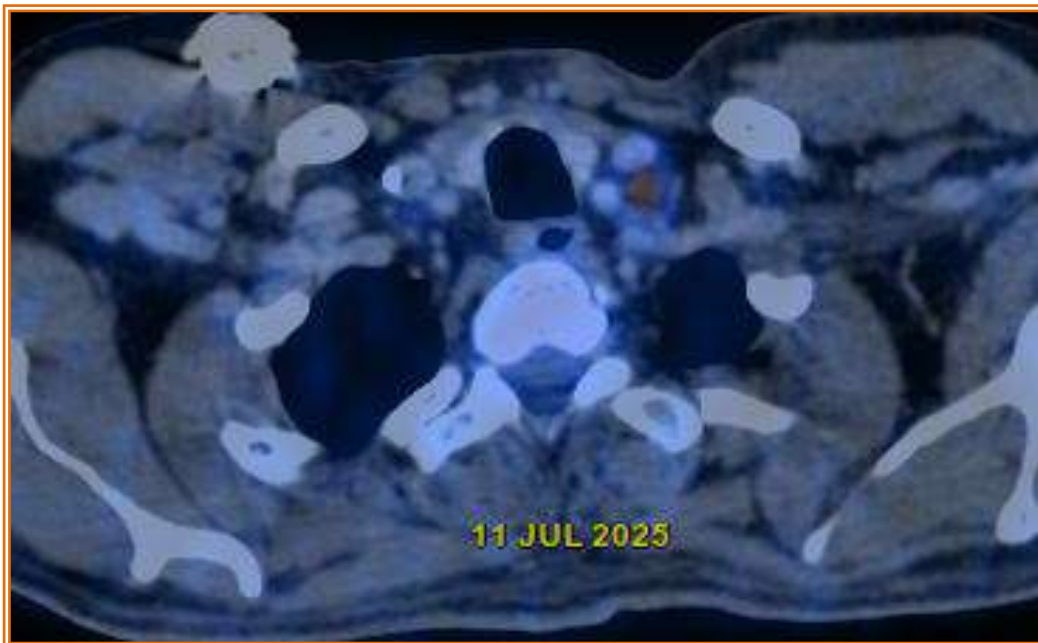
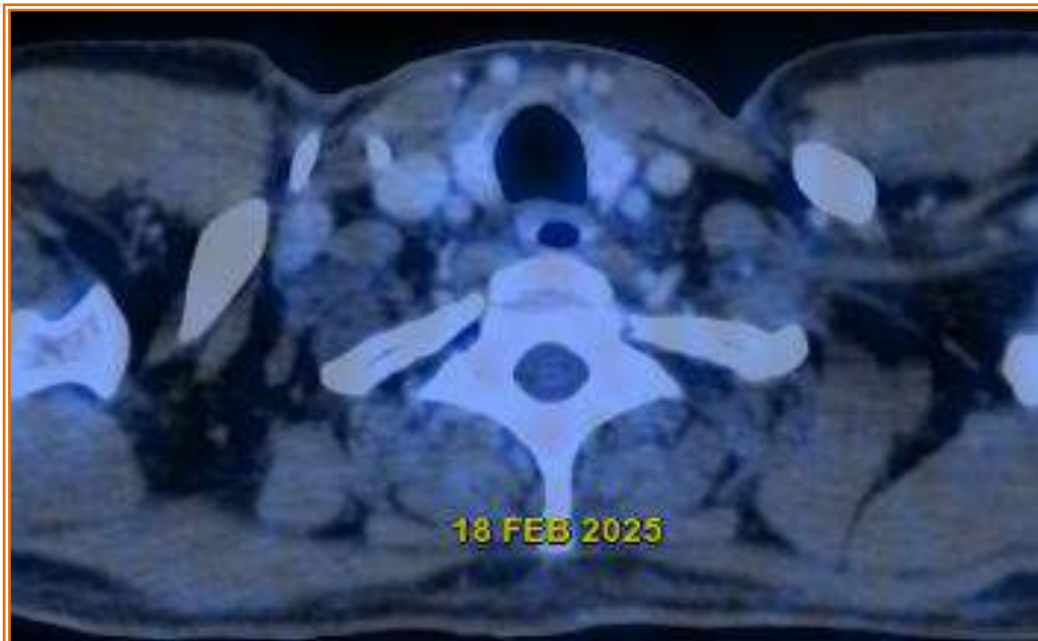
The thyroid gland appears normal.

PETCT Chest:

Appearance of FDG avid multiple pulmonary nodules noted in bilateral lung fields with some showing increased metabolic activity, largest measuring ~ 13mm in left lower lobe. (SUV Max = 4.8)

Appearance of few subcentimeter right epiphrenic, lower esophageal, subcarinal, right hilar and right paratracheal lymphnodes with increased metabolic activity. (SUV Max = 6.5)

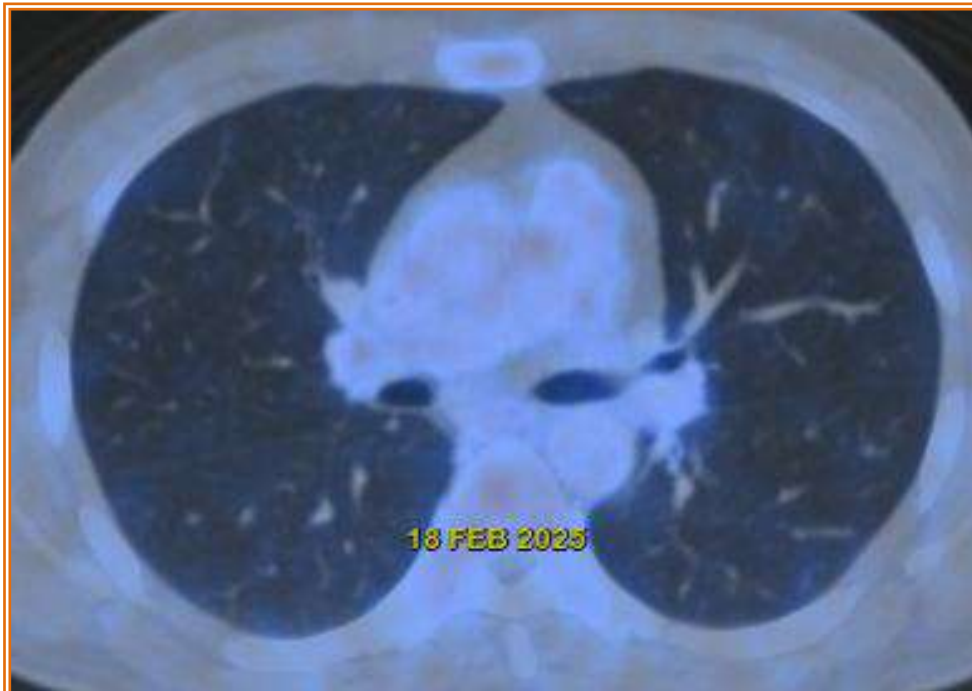
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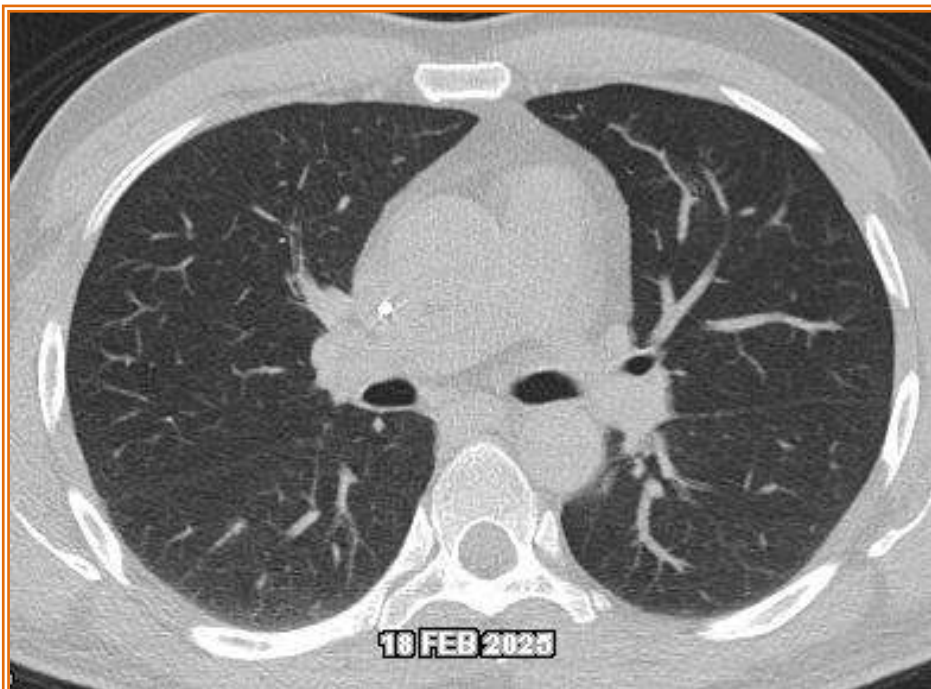
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Stable subpleural nodule measuring ~ 3 mm noted in the posterior segment of right upper lobe.

Bilateral apical fibrosis noted.

No evidence of significant pulmonary nodules

Chemoport in situ.

No evidence of pleural effusion or pneumothorax.

Lower trachea and main bronchi are unremarkable.

CT Brain:

The cerebral hemispheres, brainstem and cerebellum appear normal.

No evidence of midline shift.

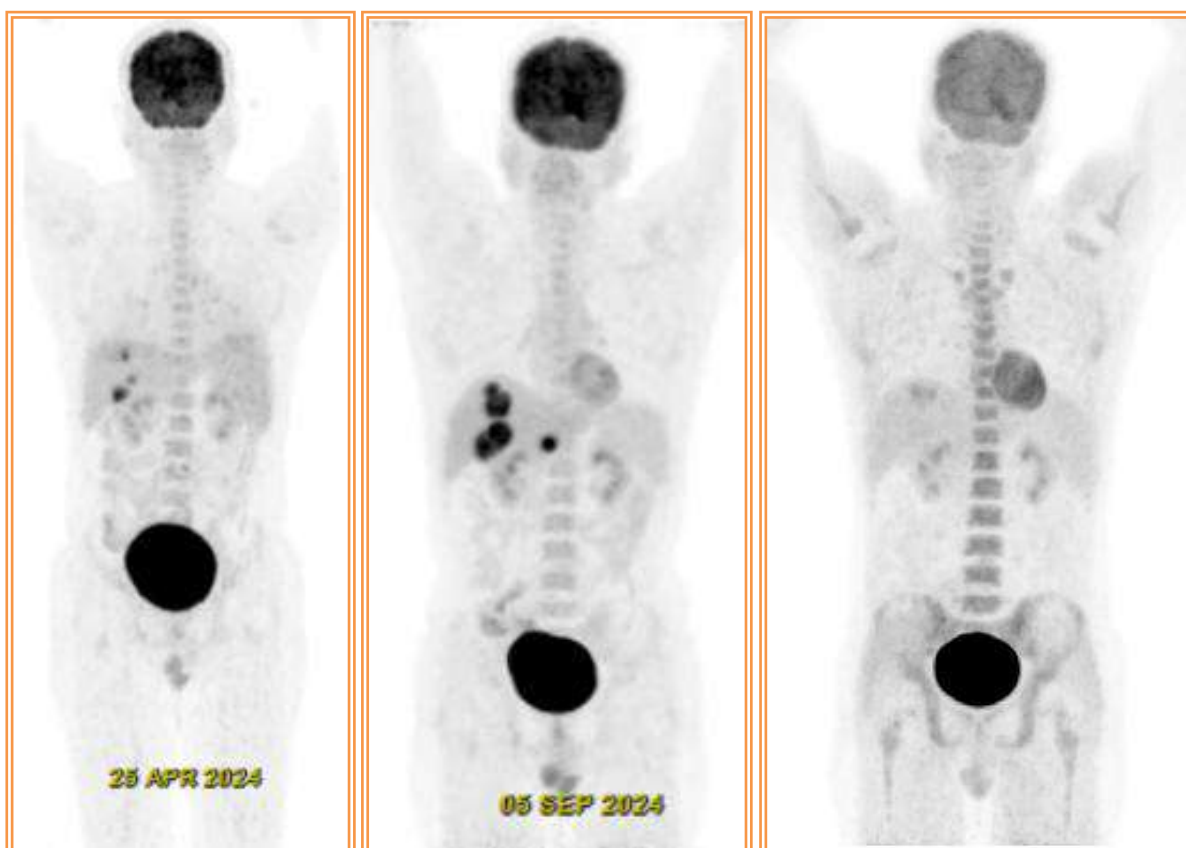
The ventricles, sulci and basal cisterns are unremarkable.

There is no evidence of abnormal increased FDG accumulation in any other visceral organs or elsewhere in the body.

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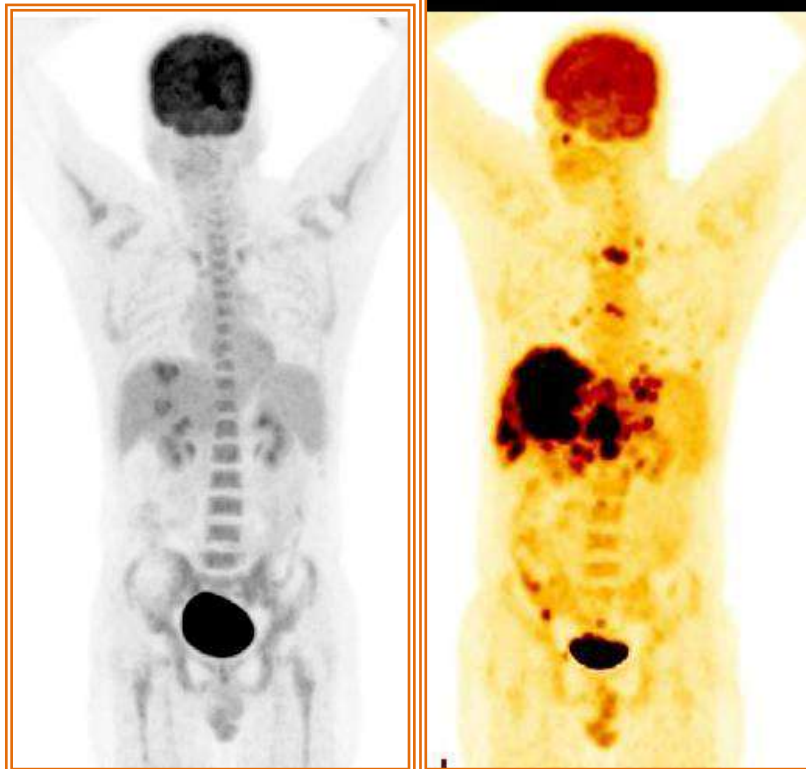
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03.01.2025



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18.02.2025



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IMPRESSION PETCT:

On comparison with previous PETCT dated: 18.02.2025

- ❖ Increase in mural thickening in the pyloric antrum of stomach with appearance of increased metabolic activity – **Active disease.**
- ❖ Increase in size of peri pyloric and left gastric lymph nodes with appearance of metabolic activity – **Active nodal metastases.**
 - Appearance of metabolically active subcentimeter celiac, peripancreatic, paraaortic, retrocaval and paracaval lymphnodes.
 - Small peripancreatic lymphnode is seen infiltrating the adjacent neck of pancreas.
 - Appearance of few subcentimeter bilateral cervical, mediastinal and right hilar lymphnodes with increased metabolic activity.
- ❖ Appearance of diffuse mesenteric and omental haziness without significant metabolic activity – *Suggested follow up to rule out early metastases.*
 - Appearance of diffuse peritoneal thickening without significant metabolic activity.
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- ❖ Increase in size, number and metabolic activity of heterogeneously enhancing hypodense lesions in both lobes of liver – **Active hepatic metastases.**
- ❖ Appearance of small T2 heterointense lesion with restricted diffusion abutting and infiltrating the neck of pancreas with increased metabolic activity.
- ❖ Appearance of metabolically active multiple pulmonary nodules in bilateral lung fields with some showing increased metabolic activity – **Active pulmonary metastases.**

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- ❖ Appearance of metabolically active multiple lytic / sclerotic lesions in right ilium, sacrum, right mandibular condyle, D2, D10, D12 and L4 vertebra – Active skeletal metastases.
- ❖ No evidence of metabolically active disease anywhere else in the body.

Comments:

- PETCT features are suggestive of progressive disease.

This report is a professional opinion from a panel of doctors comprising Dr. Srinivasaraman, Dr. Suriyakumar, Dr. Bhuvana, Dr. Vijayakrishna Chowdary and is verified by the undersigned



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